

Barcarse Psychological Services

Psychological Clinic Services
Psychological Assessment Section
Sampaloc, Manila, Philippines

PSYCHOLOGICAL / NEURODEVELOPMENTAL ASSESSMENT REPORT

I. IDENTIFYING INFORMATION:

Name :
Age :
Date of Birth :
Sex :
Residential Address :
Date of Report :

II. REASON FOR REFERRAL:

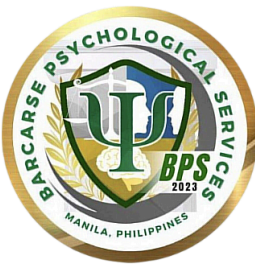
Client Frances was referred for a neurodevelopmental psychological assessment to evaluate her developmental functioning and other related concerns.

III. EARLY DEVELOPMENT AND BACKGROUND

Frances is a 10-year old female and an incoming grade 6 student. She is enrolled in a public school with honors according to her mother. The teacher of the client suggested for her to be assessed as they observed that the client isolates herself and doesn't want to interact with her classmates. The mother stated that she made the client wear a diaper in going to school up to now to avoid going to the restroom and prevent any untoward accidents. Another reason she mentioned was that the client will probably not ask her teacher to be excused to go to the rest room. The mother recalled an incident when the client was a bit younger wherein she would just urinate in her place and the client's teacher just informed her about what happened.

The client was assessed by a Developmental Pediatrician when she was three years old and underwent speech and occupational therapy but was stopped due to pandemic. Her first diagnosis was Global Developmental Delay (GDD) and the second one was Attention Deficit Hyperactivity Disorder (ADHD). The client's last therapy was speech therapy. The client's mother sought a psychological assessment due to her daughter's avoidance to interact and play with her classmates and to anyone who is the same age or younger than her. The mother wants the client to be just like normal kids who interact and play with other children her age.

The mother got pregnant with the client at 40 years old, had a normal pregnancy and did not take any medications. For the developmental milestone, the mother mentioned that the client was able to walk on time but was still unable to talk at 2 years old. The mother noted that the child of her half-sibling on her father's side has the same condition with the client.



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IV. BEHAVIORAL OBSERVATION AND MENTAL STATUS EXAM

The client presented herself in a well-groomed manner, wearing a light pink dress and carrying a backpack with a towel over her back. Her primary language is English. The client's mood was adequate and was also cooperative and responded with enthusiasm. She communicated in a stuttering manner and preferred to use words within sentences rather than defining them individually. She struggled to construct complete sentences, although she sometimes conveyed a sense of meaning, she was inclined to use incorrect words. When asked to identify what is silly or impossible in a picture, she would tell stories instead. She was able to maintain eye contact and focused on the block span. There were instances when, faced with long stories or questions, she requested that they be repeated. The client got distracted when being told long stories, fidgeting with the block or her fingers. She took longer to respond to the latter part, especially when reading words. She tended to give up on challenging and frustrating questions. When she rejoined her mother in another room, she occasionally made eye contact and responded to questions when being asked. She also initiated social interaction as needed. Though she can communicate, the lack of coherent thought when talking was observed.

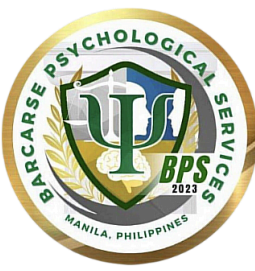
V. ASSESSMENT BATTERY

Assessment Tests and Methods	Date Administered
1. Neurodevelopmental Intake and Observation Checklist (NIOC)	May 19, 2026
2. Stanford-Binet Intelligence Scales - Fifth Edition (SB-5)	May 19, 2026
3. Adaptive Behavior Assessment System - Third Edition (ABAS-3)	May 19, 2026
4. Attention Deficit Hyperactivity Disorder Test - Second Edition (ADHDT-2)	May 19, 2026
5. NICHQ Vanderbilt Assessment Scale - Parent Informant	May 19, 2026
6. Gilliam Autism Rating Scale - Third Edition (GARS-3)	May 19, 2026
7. The Childhood Autism Spectrum Test (CAST)	May 19, 2026
8. Strengths and Difficulties Questionnaire	May 19, 2026
9. Clinical Interview	May 19, 2026
10. Behavioral Observations	May 19, 2026

VI. TEST RESULTS AND INTERPRETATION

1. Stanford-Binet Intelligence Scale – Fifth Edition (SB-5)

The Stanford-Binet Intelligence Scales – 5th Edition (SB-5) is an individually administered, comprehensive assessment tool designed to assess intellectual functioning and measure cognitive abilities across various age groups. The SB-5 consists of a series of subtests that evaluate different



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areas of intelligence, such as verbal and non-verbal fluid, knowledge, quantitative reasoning, visual-spatial processing, and working memory. The SB-5 yields several composite scores, including the Full Scale Intelligence Quotient (FSIQ), which provides an overall measure of intellectual ability. The FSIQ is composed of Verbal Intelligence Quotient (VIQ) and Non-Verbal Intelligence Quotient (NVIQ)

Table 1. SB5 test results and test-age equivalence

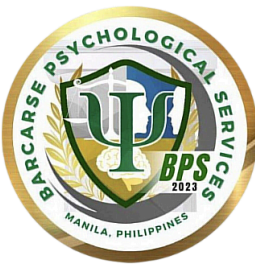
FACTOR INDEX	RESULT	AGE EQUIVALENT
FULL SCALE IQ	Mildly impaired/delayed	6 years and 8 months
Nonverbal Domain IQ	Low Average	7 years and 11 months
Verbal Domain IQ	Mildly impaired/delayed	5 years and 9 months
<i>Fluid Reasoning</i>	Borderline impaired/delayed	7 years and 4 months
<i>Knowledge</i>	Mildly impaired/delayed	6 years and 0 months
<i>Quantitative Reasoning</i>	Borderline impaired/delayed	6 years and 11 months
<i>Visual-Spatial Processing</i>	Borderline impaired/delayed	6 years and 7 months
<i>Working Memory</i>	Borderline impaired/delayed	7 years and 1 months

The client's primary language is English in responding to tasks. There are no off task behaviors observed and she answered enthusiastically. The client's Full Scale IQ falls within the mildly impaired or delayed range, corresponding to an age-equivalent of 6 years and 8 months, indicating significantly below-average general intellectual functioning.

The Nonverbal IQ falls within the low average range, corresponding to an age-equivalent of 7 years and 11 months, indicating a relatively enhanced ability to process visual-based information. Furthermore, the Verbal IQ is mildly impaired or delayed, with an age-equivalent of 5 years and 9 months, which demonstrates persistent challenges in processing language-based information despite some developmental progress.

The Fluid Reasoning scale, which measures the ability to solve novel problems, identify patterns, and analyze cause-and-effect relationships, is borderline impaired or delayed with an age-equivalent of 7 years and 4 months. This indicates difficulty grasping abstract relationships and solving problems beyond what is immediately visible.

The Knowledge scale, assessing vocabulary, factual knowledge, and recognition of absurdities, is also mildly impaired or delayed with an age-equivalent of 6 years and 0 months. This suggests the client's level of accumulated information, vocabulary, and general-knowledge learning is significantly below the average for her age, but not in the more severe range.



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The Quantitative Reasoning scale, which involves solving number-based problems and understanding numerical concepts, falls within the borderline impaired or delayed range with an age-equivalent of 6 years and 11 months. This indicates notable difficulty with numerical concepts, quantitative problem-solving, and math reasoning compared with age peers.

The Visual-Spatial Processing scale, measuring the ability to perceive and manipulate visual patterns and spatial relationships, is borderline impaired or delayed with an age-equivalent of 6 years and 7 months. This suggests relative weakness with puzzles, block designs, or visual cues.

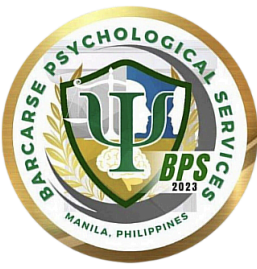
The Working Memory scale, assessing the ability to hold and manipulate information in short-term memory, is in the borderline impaired or delayed range with an age-equivalent of 7 years and 1 month. This reflects significant difficulty with tasks requiring short-term retention and mental holding/manipulation of information.

2. Adaptive Behavior Assessment System – Third Edition (ABAS-3)

The Adaptive Behavior Assessment System – Third Edition, is an individually administered measure of adaptive behavior for ages birth through 89. This measure provides a comprehensive, norm-referenced assessment of adaptive skills needed to effectively and independently care for oneself, respond to others, and meet environmental demands at home, school, work, and in the community. The ABAS-3 assesses adaptive behavior in the three broad domains of Conceptual, Social, and Practical. It provides for assessment by multiple respondents (e.g., parents, teachers, family members, supervisors, self), evaluates functioning across multiple settings (schools, clinics, hospitals, residential facilities, and community agencies), and contributes to a complete assessment of the daily, functional skills of an individual.

Table 1. ABAS-3 Result and Age Equivalent

AREA/DOMAIN	RESULT	AGE-EQUIVALENT
GLOBAL ADAPTIVE COMPOSITE (GAC)	Extremely Low	
Conceptual Domain	Low	
<i>Communication</i>	Low	Less than 5 years
<i>Functional Pre/Academics</i>	Average	9 years, 4 months to 9 years, 7 months
<i>Self-Direction</i>	Low	5 years, 4 months to 5 years, 7 months
Social Domain	Extremely Low	
<i>Leisure</i>	Extremely Low	Less than 5 years
<i>Social</i>	Below Average	5 years, 8 months to 5 years, 11 months
Practical Domain	Extremely Low	
<i>Community Use</i>	Average	10 years, 8 months to 10 years, 11 months
<i>Home Living</i>	Below Average	6 years, 8 months to 6 years, 11 months
<i>Health and Safety</i>	Extremely Low	Less than 5 years



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Self-Care	Extremely Low	Less than 5 years
Work	N/A	N/A

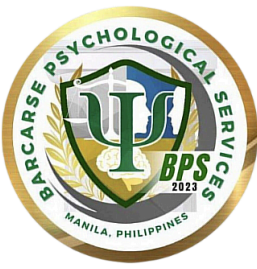
With all adaptive skill areas considered, client Frances's **overall adaptive functioning** falls within the **extremely low** range. This indicates that there is a significant delay in her adaptive functioning compared to children similar to her age. In the **Conceptual Domain**, her functioning is also within the **low** range, reflecting significant delays in her development in communication, functional academics, and self-direction. Specifically, her **communication** skills are in the **low** range with an age equivalent of less than five years, suggesting difficulties in her expressive and receptive language abilities. Her **functional pre-academic** skills are in the **average** range with an age equivalent of 9 years, 4 months to 9 years, 7 months, indicating her adequate literacy and numeracy skills. Her **self-direction** skills are in the **low** range with an age equivalent of 5 years, 4 months to 5 years, 7 months, reflecting inadequate ability to manage behavior, make decisions, and complete tasks independently.

In the **Social Domain**, client Frances's adaptive functioning is within an **extremely low** range, highlighting notable delays in social and leisure skills. Her **leisure** skills are in an **extremely low** range with an age equivalent of less than 5 years, suggesting major delays in engaging in play, recreation, and personal interests. Her **social** skills are in the **below average** range with an age equivalent of 5 years, 8 months to 5 years, 11 months, reflecting minimal ability to relate to others emotionally and behaviorally.

In the **Practical Domain**, client Frances's functioning is within an **extremely low** range, underscoring weak abilities in everyday adaptive tasks. Her **community use** skills are in the **average** range with an age equivalent of 10 years, 8 months to 10 years, 11 months, indicating the ability to navigate and function independently in community settings. Her **home living** skills are in the **below average** range with an age equivalent of 6 years, 8 months to 6 years, 11 months, reflecting minimal delay to perform household responsibilities. Her **health and safety** skills are in an **extremely low** range with an age equivalent of less than 5 years, suggesting major delay in recognizing hazards and ability to maintain personal safety. Finally, her **self-care** skills are in an **extremely low** range with an age equivalent of less than 5, reflecting considerable difficulty to maintain hygiene and perform basic self-care routines independently.

3. Attention Deficit and Hyperactivity Disorder Test - 2 (ADHDT-2)

The Attention-Deficit Hyperactivity Disorder Test - Second Edition (ADHDT-2) helps in quickly identifying ADHD in clients and estimating its severity. It meets diagnostic criteria for ADHD published in the Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition. 33



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clearly stated items are grouped into two subscales - Inattention and Hyperactivity/Impulsivity - and describe the characteristic behaviors of clients with ADHD.

The results of the ADHDT-2 as reported by the parent indicate that the client reflects a **likely presentation of Attention-Deficit/Hyperactivity Disorder**. On the inattention subscale, responses suggest occasional difficulties with sustaining attention. Similarly, on the hyperactivity/impulsivity subscale, scores reflect some presence of impulsive or active behaviors. These results suggest mild concerns in attention and activity regulation that should be interpreted with caution and considered alongside other data.

4. NICHQ Vanderbilt Assessment Scale - Parent Informant

The NICHQ Vanderbilt Assessment Scale - Parent Informant is a tool used to assess symptoms of Attention-Deficit/Hyperactivity Disorder (ADHD) in children and adolescents, and to screen for other co-occurring conditions like oppositional defiant disorder, conduct disorder, and anxiety/depression.

Table 2. Vanderbilt Assessment Scale Interpretation of Results

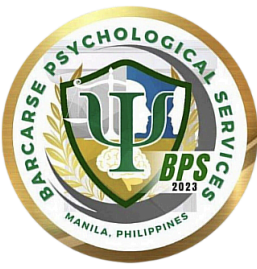
SUBSCALE/DOMAIN	INTERPRETATION
ADHD (Inattentive subtype)	Present
ADHD (Hyperactive subtype)	Not Present
Oppositional Defiant Disorder	Present
Conduct Disorder	Not Present
Anxiety and/or Depression	Not Present

Based on the results of the Vanderbilt ADHD Diagnostic Rating Scale (Parent Version), the client's scores indicate that most behavioral domains are within normal limits except that manifestations of ADHD are present in Inattentive and Oppositional Defiant Disorder subtypes.

5. Gilliam Autism Rating Scale - Third Edition (GARS-3)

The Gilliam Autism Rating Scale - Third Edition (GARS-3) is a standardized rating form that accurately identifies children with autism spectrum disorder. The rating form consists of 56 items grouped into six subscales: Restrictive/Repetitive Behaviors, Social Interaction, Social Communication, Emotional Responses, Cognitive Style, and Maladaptive Speech.

The client's profile included some noticeable variations in restricted or repetitive behaviors, social interaction, social communication, emotional responses, cognitive style, and speech patterns. The client's presentation is consistent with **Very Likely Autism Spectrum Disorder (Level 2) Requiring Substantial Support**, indicating significant autistic qualities that may affect social



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communication and restricted/repetitive behaviors. These findings mark deficits in verbal and nonverbal social communication skills; social difficulties persist even with support systems in place; limited initiation of social interactions; and reduced or unusual responses to social advances from others. Within the scope of restrictive/repetitive behavior, routines are often essential rather than preference; substantial discomfort follows when they are interrupted. For families and support teams, the tailored support strategies for Level 2 autism generally include intensive speech and language therapy, Applied Behavior Analysis, organized social skills programs, and assistive communication technology. Educational environments typically require an Individualized Education Plan that includes specific classroom accommodations.

6. *The Childhood Autism Spectrum Test (CAST)*

The Childhood Autism Spectrum Test (CAST) is a screening tool designed to identify children who may have developmental differences in social and communication functioning consistent with Autism or a related condition.

Table 3. Interpretation of CAST Result

VERBAL INTERPRETATION
Significant Developmental Differences

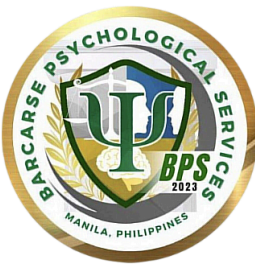
The child's CAST result falls within the range of Significant Developmental Differences, suggesting that key areas of social understanding and communication differ greatly from what is expected for the child's age. This indicates developmental concerns that may benefit from structured support, targeted interventions, and guided social interaction.

7. *Strengths and Difficulties Questionnaire (SDQ)*

The Strengths and Difficulties Questionnaire (SDQ) is a globally recognized instrument for assessing the mental health status of children and young people. It provides an overview to help detect mental health issues; however, it does not provide a clear-cut screening.

Table 4. Categorical Interpretation of SDQ Results

SCALES	CATEGORICAL INTERPRETATION
Conduct Problems	Close to average
Hyperactivity Scale	Close to average
Emotional Problems	High
Peer Problems	Very high
Prosocial Scale	Very low



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Externalizing	Close to average
Internalizing	Very high
Total Difficulties	Slightly raised

Based on the client's profile, she has close to average scores in the areas of Conduct Problems, Hyperactivity, and Externalizing. High in Emotional Problems domain, very high in Peer Problems and very low in terms of Prosocial Scale. This indicates the client falls within the normal range, meaning she displays typical behaviors for her age and peer group. Exhibits age-appropriate behavior, rules adherence and emotional regulation. Her attention span, impulse control and activity levels are typical. Also, the result also confirms that the client is not acting out, displaying aggression or struggling with disruptive behaviors in a way that stands out from typical development. Likewise, a very high score in Internalizing and Peer Problems suggests significant emotional distress, social isolation, and a high likelihood of internalizing disorders like anxiety or depression. It serves as a red flag that she may need further clinical evaluation or support. A very low score in Prosocial Scale suggests a notable deficit in positive social behaviors. This means the client struggles with empathy, sharing, helping others, and spontaneous acts of kindness, indicating a higher risk for social isolation or peer relationship problems.

Lastly, a "slightly raised" Total Difficulties Score (TDS) on the Strength and Difficulties Questionnaire (SDQ) is an early indicator of potential emotional or behavioral challenges. It typically acts as a caution sign that needs observation or slight interventions, rather than a conclusive identification of a mental health condition.

VII. FINDINGS

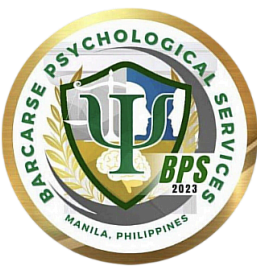
Based on the mother's reported developmental history and test results, the client meets the diagnostic criteria for a Neurodevelopmental Disorder, specifically **Autism Spectrum Disorder (Level 2)** at the time of the assessment. The information provided indicates the presence of persistent developmental delays or functional impairments across settings that would warrant such a diagnosis. However, the client's low social functioning may be attributed, in part, to her tendency to avoid or limit initiation of interaction with kids her age.

VIII. RECOMMENDATIONS

Based on the findings, the following **interventions** are recommended for the client:

Educational Placement/Intervention

- Frances Mary is recommended to be enrolled in a Special Education (SpEd) / Special Needs Education (SNEd) program for a structured environment with a low student-teacher ratio,



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individualized instruction, and behavioral support. It is essential for Frances Mary's learning environment to be tailored to her developmental profile, considering Autism Spectrum Disorder (ASD). Instruction should emphasize structured, interest-based, and language-enriched activities that provide clear guidance for completing simple to complex tasks, promote sustained attention, and include explicit behavioral expectations.

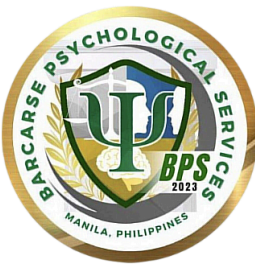
- Teachers are encouraged to implement Individualized Education Program (IEP) such as: allowing extra time for tests and assignments, breaking larger projects into smaller, manageable steps, providing visual support by incorporating pictures, symbols, or written words to help explain information or routines, using clear and concise instructions, and arranging seating to minimize distractions while keeping Frances Mary close to the teacher for support. The learning environment should incorporate guided play, structured routines, and consistent opportunities for positive peer interaction.
- In addition, targeted social skills instruction should be included to help Frances Mary build meaningful connections, improve communication, strengthen impulse control, and develop adaptive skills for group settings. Reinforcement strategies and consistent routines can help manage attention difficulties in structured learning contexts.

Therapeutic Interventions

- **Applied Behavioral Analysis (ABA):** It is highly recommended for Frances Mary to undergo Applied Behavioral Analysis (at least three times a week) in order to address her impairment in terms of interaction skills such as communication and socialization as well as to decrease the frequency of her behaviors that interferes with her learning and functioning. This technique is an evidence-based approach to build communication, reduce challenging behaviors, and teach adaptive skills.
- **Occupational Therapy (OT):** Frances Mary is encouraged to attend Occupational therapy (at least twice a week) to enhance Frances Mary's independence in essential daily living skills and other self-care routines. OT should address sensory processing, fine motor skills, and self-regulation, thereby improving participation across home, school, and community settings.
- **Speech and Language Therapy:** Speech therapy (at least twice a week) is highly recommended to improve Frances Mary's focus on pragmatic language, conversational skills, and comprehension. Therapy should focus on sentence formation, expanding vocabulary and generalization of communication skills across multiple settings.
- **Social Skills Training:** Frances Mary will benefit from small group sessions to practice peer interaction, emotional recognition, and cooperative play. It provides opportunities for children with ASD to practice social skills in a structured environment.

Family Support

- Maintain a safe, supportive, and nurturing home atmosphere where Frances Mary feels secure



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and understood. Establish and sustain consistent daily routines to foster predictability, which can assist in alleviating anxiety and intense emotional reactions.

- Parents are encouraged to receive structured guidance on integrating communication, play, self-help skills, and behavioral strategies into everyday routines. Engaging in training programs can equip caregivers with the necessary skills to address behavioral challenges and enhance communication skills with the child. Furthermore, having a strictly followed screen time schedule partnered with interactive play, hands-on activities, and shared experiences will support language development, problem-solving, social engagement, and self-regulation.

Monitoring and Follow-Up

Monitoring and follow-up assessment should be structured, consistent, and multidisciplinary to ensure the client's educational, behavioral, and developmental needs are tracked over time. Continuing collaboration among parents, teachers, therapists, and specialists every 6-12 months is highly recommended for comprehensive reassessment of social, communication, behavioral, and adaptive skills. For re-evaluation of ASD severity level to determine if support needs have changed (level 1, 2 or 3), every 1-2 years is suggested.

Prepared by:

Clinic Intern

Barcarse Psychological Services—Psychological Assessment Section

Reviewed by:

Name of Psychometrician

Psychometrian

Barcarse Psychological Services—Psychological Assessment Section

Psychometrician License No.:

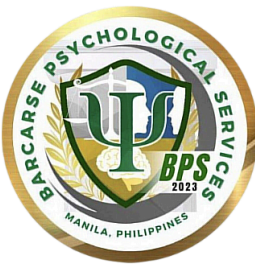
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Approved by:

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